VARIOUS LOGOS

Provider Initiated HIV Testing and Counseling Training Programme

Facilitator Guide

MOH/NHP/ CHART/CHAI
2013

INTRODUCTION

Intended Participants

This training course is targeted to health care providers who are charged with the responsibility for establishing and implementing routine provider-initiated, opt-out approach to HIV testing and counseling. This training is tailored for health care providers in the public hospitals and Health Centre

It is assumed that individuals participating in this training would have been sensitized to HIV/AIDS basic issues, knowledge of public health system in their country, and a role in the health care delivery system where they would likely be in a position to recommend HIV testing to patients they perceive as being at risk.

Objectives

By the completion of this training, participants will be able to:

- Discuss benefits of HIV testing with patients;
- Provide brief, targeted health education on HIV prevention and transmission, as well as HIV treatment and care;
- Identify need for confidentiality and consent and how to employ these in practice settings; and

  Deliver HIV test results to the patient and reporting to the National HIV/STI Programme (NHP)

Background and Rationale

Since HIV antibody testing first became available, WHO has advocated for persons at risk for HIV to voluntarily seek out HIV testing and counseling. The cornerstone of WHO's guidance on HIV testing has remained constant for nearly twenty years: confidentiality, informed consent, and access to quality counseling. Throughout the World, programs offering voluntary counseling and testing (VCT) have been successful in many countries in providing individuals with knowledge about HIV, prevention measures, and in providing HIV test results to millions of individuals. Still, in Jamaica, up to 50% of individuals estimated to be infected are not aware that they are infected with the HIV virus. Reaching individuals living with HIV who do not know their serostatus must clearly be a global public health priority.

In light of this, coupled with advances in treatment and care, and following the WHO and UNAIDS, The Jamaican NHP has advocated for an increase in Annual Routine opt-out, provider-initiated HIV testing and counseling (PITC) at all levels of healthcare delivery in the public and private settings. The recommendation for *universal systematic* offer of HIV testing and counseling is seen as an important step to achieving the goal of universal access for all persons with HIV/AIDS. This intervention provides and entry point in to HIV treatment, care and support, behavior change counseling and provides a seminal opportunity for HIV prevention.

It should be noted that the position of WHO/UNAIDS on provider-initiated HIV testing and counseling describes two discreet categories:

- **Diagnostic Testing:** refers to situations where a medical condition [e.g., TB] or medical symptoms [e.g., OIs or unexplained significant weight loss] indicate a significant possibility of underlying HIV disease; and

- **Routine Offer:** refers to offering in settings where potential benefit is especially high [antenatal settings] or where a high probability of HIV infection exists [e.g., high prevalence regions, STI treatment settings].

An important caveat in the shift to provider-initiated testing is the importance of having mechanisms in place to assure referral to post-test counseling services to all patients, and referrals to medical and psychosocial support for those testing positive. In addition, moving to an 'opt-out' model of obtaining consent (e.g., "Unless you object, I plan to administer an HIV test) removes many of the sociocultural barriers that often impede access HIV testing services and engagement with the Healthcare system.. **Every effort must be made to ensure voluntary informed consent to HIV testing.** WHO/UNAIDS have consistently refuted any benefit to mandatory testing, and ensuring consent is a principle that cannot be overlooked.

Finally, health workers should remain aware of the power--and potential status--of the health care provider in their setting and in their community. The recommendation for HIV testing will be, for some patients, tantamount to an edict. Sensitivity to patient volition and recognition of the right to refuse testing should guide a provider's interactions in obtaining consent.

When moving to routine offer, the standard pre-test counseling components used in VCT are adapted to simply ensure informed consent, without a full education and counseling session. However, it is an expectation that every effort will be made to identify additional support within the health care setting for education and emotional support as part of the -HIV post-testing interaction for those who may exhibit/request additional support requirements.

Finally, this course does not attempt to meet the needs of all clinicians doing routine offer testing. WHO and the U.S. CDC have jointly produced curricula that deal with testing in TB and ante-natal settings. The primary utility of the materials in this training package are for clinicians treating STIs, providing family planning services, and in general medical settings with high HIV prevalence. It is also important to acknowledge that this course does not deal with some important clinical information which would need to be part of a course on diagnostic testing for suspicion of HIV infection. The training that most closely meets that need would be the Integrated Management of Adult/Adolescent Illness (IMAI) courses developed and delivered by WHO.

Goal of course

A primary goal for this course is to increase the capacity of health care providers to offer HIV testing to patients seeking care in public hospital and health Centre facilities.

In the development of these materials, an important focus has been on adapting the model of HIV testing and counseling to:

- address health care providers' concerns about time and staff constraints in busy health care delivery settings;
- support the implementation of an approach to ensure that a greater number of persons know their status and access treatment; and

- offer clear and direct suggestions on achieving goals for provider initiated opt-out testing and counseling.

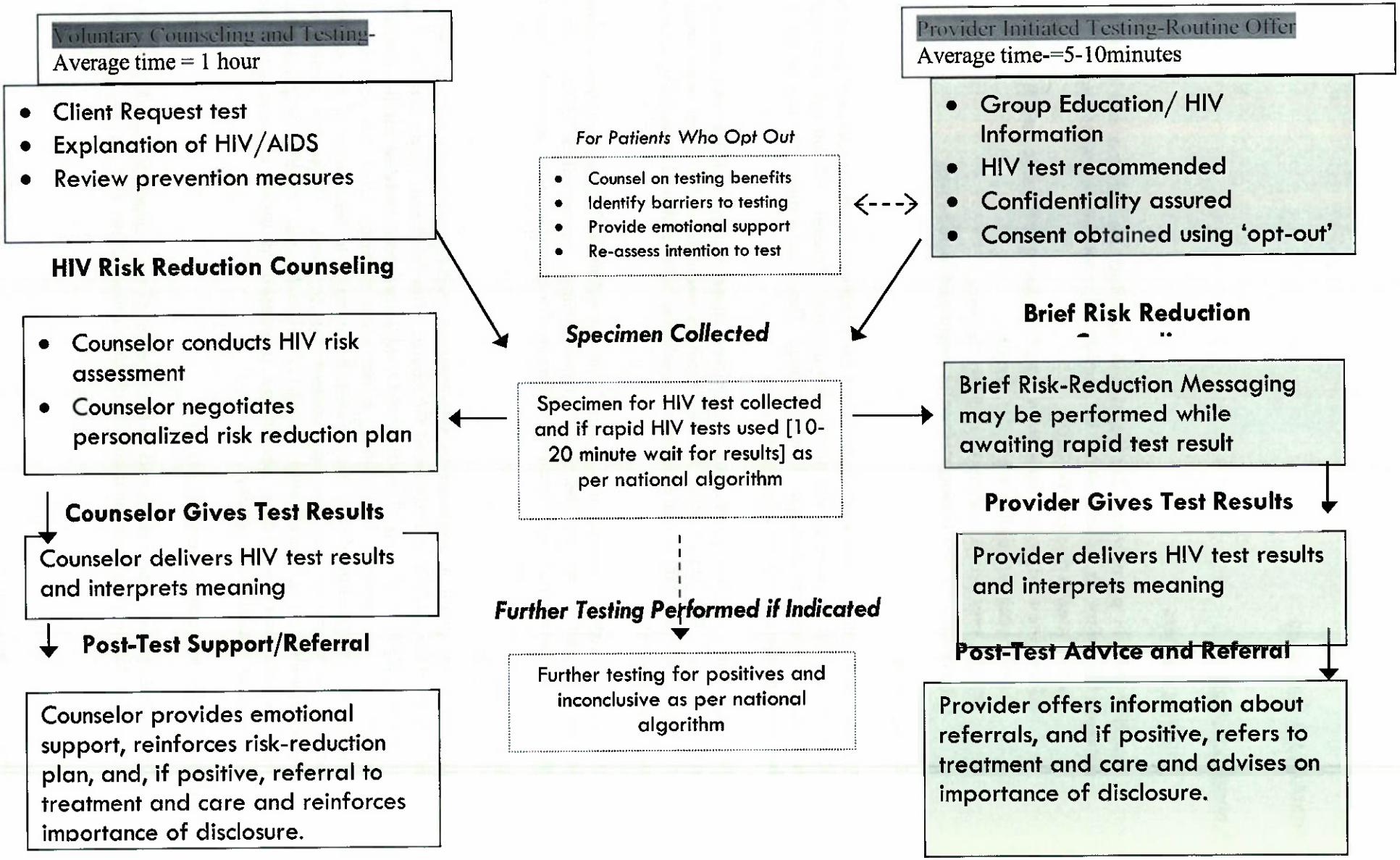
This training has been designed to offer a model (see overleaf) of HIV testing in which the provider recommends HIV testing with 'opt-out' language, assures confidentiality, and obtains informed consent. The model offers a graphic comparison between a typical VCT intervention and a typical PITC intervention.

The pre-test information and education given may be offered by any member of the health care team. In this model, the education session facilitates the patient making an informed decision. The provider then confirms the patient's desire to test. Finally, the test is performed and results are delivered with post test counseling focused on providing access to treatment and partner counseling services if positive, and with any necessary referrals. The provider-initiated opt out approach yields a high uptake in testing. Critical to this process however is providing the patient with an opportunity to opt out if they so desire, though providers should assess patient's barriers to testing and create a plan with the interdisciplinary team to overcome this resistance.

In Jamaica, the NHP has adopted the use of Rapid HIV testing, utilizing validated algorithms that allow for same-day testing and delivery of confirmed HIV status to the patients. The PITC opt-out approach is a key intervention to improving coverage rates of HIV screening, increased engagement in HIV-related services and reducing healthcare burdens associated with late-stage HIV diagnosis.

This course is intended to enable the provider to understand and deliver the provider-initiated approach to HIV testing and counseling.

COMPARING PROVIDER-INITIATED AND VOLUNTARY HIV TESTING AND COUNSELING



Adapting this Curriculum

Preparing to Train

1. Selecting Trainers

The first step in preparation for training is to select the appropriate staff to act as trainers. The following criteria should be considered in making this selection:

- Expert Knowledge of HIV/AIDS
- Good knowledge and understanding of the Jamaican Public Health system
- A role within the delivery of care system
- History and proven ability at training delivery.
- Knowledge of HIV testing strategies employed in Jamaica.

2. Trainers Role

Trainers who utilize these materials should be familiar with the facilitator guide and the slides which will be used at the participant manual. Trainers will be asked to both lead lectures and facilitate discussion in this training. The ideal trainers for this material would be comfortable in both teaching modalities.

Finally, trainers who are preparing to train from these materials should be both organized and flexible. The preparation for this training requires a facilitator who knows their audience, who can adjust material as needed, and who is skilled at training coordination and management.

Whenever possible, pairing one trainer with an additional trainer provides benefits to learners as well as making the task of training more manageable. Using two trainers makes any educational experience for engaging for learners and is easier for trainers.

3. About the guide

The facilitator guide is designed to help trainers guide participants through lectures and exercises which are the foundation of this course. The facilitator guide is divided into four modules. At the beginning of each module, expected time needed for the module to be delivered is introduced. In addition, a narrative description of the objectives for the module and suggestions for preparation are outlined. In the course of the module, sub-modules with specific activities and content are outlined. When the content calls for didactic presentation, a numbered outline is to be followed. When prompted by a bullet with checked box, the trainer should be prompted to either engage participants in discussion or facilitate an activity.

4. The Participants' Manual (companion piece)

The participant manual is designed to be a 'map' of the training for participants and also a tool for future reference in their HIV testing interactions. The participant manual has

been created using Power Point. In preparing the materials, you should print copies as handouts spaced 3 per page.

The slides are designed for use as slide or overheads, but, for the most part, can easily be adapted and placed on flip charts. Again, these have been created in Power Point and have been numbered for easy reference. References in facilitator guide to slides correspond to slide numbers in the slide set. Note that the participant guides have been created with additional space for notes, so participants should be encouraged to make additional notes.

MODULE 1: INTRODUCTION TO TRAINING AND OVERVIEW

Total Time: 30 minutes

10 minutes	Welcome and Introductions
5 minutes	Course Objectives
15 minutes	Introducing the Topic: Provider-Initiated HIV Testing

Module Objectives: Participants will be introduced to one another, and the trainer will introduce herself/himself and the training content. Overview, agenda and objectives for the course will be presented, and the rationale for provider-initiated testing and counseling will be explained. A brief overview for PITC protocol will be offered.

Preparation: Participant manuals should be prepared. If slides are to be shown, projector with back-up bulb should be ready. A flipchart and newsprint should be available for notes and lecture points. If a slide projector is not used, writing most notes from slides onto newsprint sill suffice.

WELCOME AND INTRODUCTIONS

Time: 10 minutes

Materials: Flipchart

Steps:

1. Welcome participants to the training. Thank them for their willingness to participate and openness to learning. Emphasize that HIV is an important problem, and everyone in their community is lucky to have caring health care workers willing to respond.

2. Briefly discuss logistics for the training. State that a full training day is planned, and attendance all day is necessary for participants to learn what they will need to know. Explain there will be tea breaks in the morning and afternoon, and discuss options for lunch. Make sure everyone is informed where the lavatories are, and ask that all mobile phones be turned off to minimize distractions during the day.

☑ *Introduce yourself and, if applicable, your co-trainer. Emphasize your experience in training and HIV testing and counseling as well as other relevant health care experience. Ask participants to introduce themselves briefly to the group, noting that there will be other opportunities through the day to continue finding out more about one another. Ask participants to mention the following points:*

- Name
- Work setting
- Challenges associated with HIV testing

3. As participants introduce themselves, model effective listening and convey interest and empathy. If anyone takes too long, remind them that this exercise is designed to be brief.

☑ *As people are stating the challenges to HIV testing, record these on prepared flip chart labeled "Challenges".*

4. Once introductions are complete, acknowledge the important experience and compassion of participants, and advise them that they can help make this a valuable learning experience by offering solutions they have found in addressing some of the challenges which are part of HIV testing and HIV care provision. If participants disclose that they are living with HIV/AIDS, stress that they are an extraordinary resource in this training and to patients and thank them for their disclosure.

5. [Optional] You may wish to generate a list of ground rules which you will ask participants to adhere to. If you do, begin by labeling a flip chart, then ask for volunteers to suggest ground rules to be followed by you and members of the learning community during the training. As participants make suggestions, record these to newsprint. Hang these in a prominent location in the training room.

COURSE OBJECTIVES/AGENDA

Time: 5 minutes

Materials: Agenda (Slide 1), Course Objectives (Slides 2), Protocol for PITC (Slide 3)

Steps:

☑ *Reveal Slide 1, and ask refer participants to turn to the agenda in participant manuals.*

1. Review structure for the day, inviting any questions about content as you proceed. Offer the suggestion that the day is a full day of training and your hope that it will be rewarding. Suggest that the curriculum has been developed to compromise one day of training factoring in an hour lunch break and morning and afternoon tea break.

☑ *Reveal Slide 2, and discuss objectives for the course. By the completion of this training, participants will be able to:*

- Discuss benefits of HIV testing with patients who are offered HIV tests;
- Provide brief, targeted health education on HIV, transmission and prevention, and access to treatment and care;
- Identify need for confidentiality and informed consent, and understand how to employ these in their practice settings; and
- Deliver HIV test results, including referral to treatment and care, prevention support, as well as potential need to notify partners.

☑ *Reveal Slide 3, and suggest that this depiction summarizes one possible protocol for a provider-initiated test intervention. Quickly review the protocol making the following points:*

- The intervention begins with either some form of group education or with a provider recommendation including brief HIV content. Note the traditional VCT pre-test components are abbreviated.
- Consent is obtained, assuring confidentiality.
- Sample is obtained for HIV testing.
- Results are given and interpreted.
- Post-test referrals and support provided. Counseling may or may not be offered in this setting; a referral to quality counseling may replace traditional post-test counseling in VCT.

Offer this as a BRIEF introduction to PITC, and note that this the rest of the course will explore each step in the intervention in great detail.

INTRODUCING THE TOPIC: PROVIDER-INITIATED HIV TESTING

Time: 15 minutes

Materials: Flipchart; Addressing Concerns in PI Testing (Slides 4 & 5)

Steps:

1. Remind participants of the title of the course. Ask their thoughts on why a course was developed that focused on the health care worker recommending HIV testing to a patient who has not requested the test. If they need another prompt, ask what percent of their patients are at risk for HIV. Next ask what percent of patients see themselves at risk. Listen and confirm responses. If no one has mentioned the point, be sure to emphasize that, in many health care settings, patients are probably more at risk for HIV than they acknowledge.

2. Throughout the Caribbean there is an urgent need to ensure that people living with HIV have access to treatment, care, support and prevention. This is not possible if the persons infected, and the health care system that serves them, are not aware of their HIV status and their needs. Health Care systems need to adopt an efficient and effective system for increasing the number of persons who have the opportunity to do an HIV test. There is a need therefore to consider a shift to the Provider Initiated Approach to achieve a high uptake in HIV testing and counseling. This method has been used in public health settings to achieve this objective of high yield in cases of other health conditions; such as reducing syphilis in pregnant women, increasing immunization, prevention of various cancers.

☑ *Refer to the flipcharts with recorded participant concerns about HIV testing. Inform them that this course has been designed in hopes of addressing some of their concerns.*

☑ *Show Slides 4 & 5, using the following to guide your talking points about the benefits of PITC:*

Time: Begin by stressing that one important task in moving to P-I testing is recognizing the limits of Healthcare Worker time in busy medical settings. To

address this, the recommendation is that counseling expectations in VCT be waived.

Human Resources: One option for delivering high-quality testing and counseling while addressing the time constraints for health care workers is to use other staff to assist with HIV testing tasks. These may include existing staff and trained lay counselors.

Stigma: One important reason providers may resist offering HIV testing is patient discomfort. If patients feel 'singled out' or somehow stigmatized because HIV testing is offered, it may hurt the patient-provider relationship. If, on the other hand, this service is routinely offered to all or most patients in a setting, the procedure and offering of the procedure becomes normalized. This also serves to change community norms about HIV testing as a routine component of medical care.

Multiple Patient Needs: Some providers may be resistant to offering HIV testing because of a perception that they were to 'fix' all of the emotional and other problems facing patient. With a de-emphasis on counseling and emotional support, the testing intervention becomes a more typical medical screening procedure and linking patients to appropriate counseling and support becomes a function of effective referral not intensive individual counseling in the health care setting.

3. State that, while no intervention can address all these barriers, significant effort has been made to develop a protocol that is sensitive to concerns of health care workers and the needs of persons living with HIV who do not know their status. In addition, note that any effective HIV prevention intervention must be very attentive to patient's issues and concerns. Inform the group that the next activity is designed to think about HIV testing from the perspective of the patient.

4. Invite any final questions about the agenda or objectives and proceed to the next section by suggesting the course will turn to the question of why HIV testing is recommended-and some challenges for patients who are taking HIV tests.

END OF MODULE 1

MODULE 2: BENEFITS AND BARRIERS TO TESTING

Total Time: 50 minutes

10 minutes	Benefits of HIV Testing
5 minutes	Barriers to HIV Testing
5 minutes	Patient-Centered Care
15 minutes	Confidentiality and Consent
15 minutes	Model for Provider-Initiated HIV Test Intervention

Module Objectives: Participants will discuss the benefits and barriers to HIV testing. Principles of informed consent, confidentiality, and counseling availability will be discussed. A model for provider-delivered HIV testing and counseling will be introduced.

Preparation: Flipcharts and markers should be available for note-taking during group exercises. Trainers should be very familiar with the four step model for P-I testing, and may choose to post this in the training room. A copy of WHO/UNAIDS policy on HIV testing and counseling may be reproduced as a supplemental handout.

BENEFITS OF HIV TESTING

Time: 10 minutes

Materials: N/A

Steps:

☑ *Quickly divide participants into two groups or, in larger groups, divide into manageable groups but instruct half the groups to focus on each of two questions.*

1. Once groups are formed, explain that the task for the next 7 minutes will be to think about the benefits and barriers of HIV testing-**from the perspective of patients being offered HIV testing.**

☑ *Ask one half of the group(s) to think about the benefits of HIV testing. Ask, "If you were a patient thinking about taking an HIV test, what might you see as the good things about that medical information?"*

☑ *To the other group(s), invite them to generate a list of barriers to HIV testing. Ask, "If you were a patient who was offered HIV testing, what might you see as the bad things about taking an HIV test?"*

2. Tell groups to brainstorm as many benefits and barriers as they can in the allowed time. Give groups a two-minute notice after five minutes, and end discussions at 7 minutes.

3. Begin debriefing by asking the first group to discuss all the benefits of HIV testing they came up with. Listen and confirm responses.

☑ *If you have multiple groups, invite one group to share 2-3 ideas, and then allow other groups to add ideas until all suggestions have been offered. You should expect to hear answers like:*

- early access to treatment and care
- ability to make family planning choices
- possibility to make lifestyle changes
- ability to change behavior to avoid transmission to partners
- planning for possible health problems

4. Affirm all answers and thank the group for their suggestions. Add any additional benefits to HIV testing you can think of.

5. State that, particularly where time and resources may be limited, there may be reasons to prioritize patients who are 'most in need' of HIV testing.

☑ *When that is the case, ask participants which patients are particularly in need of provider-initiated HIV testing. If a second prompt is needed, ask what criteria they would use to determine which patients should be offered HIV testing. Expect responses like:*

- STI patients
- patients with TB
- patients with symptoms of HIV
- patients with multiple partners/risk behaviours
- women in ante-natal clinics
- Sex Workers
- MSM

6. Acknowledge the group's work, adding any priority populations not identified by the group. Invite any questions. Acknowledge their ability at understanding risk factors for HIV, but highlight that anyone is at risk for contracting HIV. Emphasis the lack of ability to identify PLHIV especially in the early stages of the disease and hence the requirement of routine annual screening.

BARRIERS TO HIV TESTING

Time: 5 minutes

Materials: Flipchart

Steps:

1. Acknowledge that health care workers are usually very familiar with the benefits of HIV testing, and sometimes not as sensitive to the barriers to testing. Inform the group that this is the purpose for including a discussion of barriers to HIV testing.

2. Ask the second group to offer the barriers to HIV testing that they identified. Again, if there are multiple groups, alternate between groups to share reporting time.

☑ *Record these on flipchart labeled "Barriers".*

3. As before, it is important to listen attentively and affirm responses.

☑ *Once this group has finished, ask remaining participants what else they would add to the list. Confirm and record responses. You should expect responses like:*

- fear of abandonment
- fear of violence
- loss of job
- loss of family support
- community rejection
- fear of illness/mortality
- denial of past HIV risk behavior
- Fear of Discrimination
- Loss of self worth

4. Conclude brainstorm by asking participants why reviewing barriers to HIV testing is important. Confirm responses. If they need a follow-up prompt, ask what might happen if providers don't understand potential barriers. Again listen and confirm responses.

5. Summarize by saying that the rationale for this course is the absolute health benefit of persons at risk knowing their HIV status. That said, the realities of patients' lives and the stigma and emotions associated with HIV testing remind us that the most effective testing and counseling programs will couple provider-initiated testing and counseling with comprehensive interventions, including **referral to sensitive counseling and prevention support.**

6. **State that any HIV testing policy which addresses human rights must include what have been called the '3 C's' of HIV testing and counseling: consent, confidentiality, and access to counseling.** Point out that, while counseling should not be seen as a *requirement* in provider-initiated testing, a model HIV testing program would have additional staff available to augment the testing interaction where resources and time prohibit more extended interaction with HIV Specialist services (The CHARES Clinic) primary care providers.

7. To help patients cope with the barriers to testing, careful attention must be paid to patients' needs when recommending HIV testing. Transition to the next activity by stating that we will take a moment to explore these needs with a preliminary examination of the provider/patient relationship.

PATIENT-CENTERED CARE

Time: 5 minutes

Materials: Flipchart

Steps:

☑ *Ask participants to think for a moment about their experiences as* ***consumers*** *of health care. Ask, when they are patients, what they value in a relationship with a health care provider. If they need another prompt, ask what qualities would help them feel safe to trust their provider and comfortable talking honestly about their sexual behavior.*

☑ *Listen carefully, and record responses on flipchart labeled "Helpful Traits". You should expect responses like:*

- compassionate
- good listener
- non-judgmental
- patient
- confidential at keeping my secrets
- knowledgeable

1. Explain that these are the types of traits you are referring to when you speak of patient-centered care. These relationship factors are important in most health care settings, but are especially critical as we think about overcoming potential barriers to HIV testing. ***Retain this newsprint for an activity during Module Four.***

2. Transition to the next section by pointing out that we will spend a moment thinking about two of the three C's in the UNAIDS guidance: confidentiality and consent.

CONFIDENTIALITY AND CONSENT

Time: 25 minutes

Materials: Flipchart

Steps:

☑ *In a matter-of-fact way, tell participants that in just a minute, they will turn to the person beside them and begin to discuss one of two scenarios: a time in which they had unprotected sex and risked HIV or STD transmission, or a time they had sex and risked an unintended pregnancy*

☑ *Excuse yourself for a moment, explaining that you are looking for something in your materials. Look through your briefcase or trainer notes for a moment or two, allowing enough time for participants to reflect on what you have asked.*

☑ *After this digression, return your attention to the class and lightly explain that no one will be asked--or allowed--to discuss their sexual history. Monitor group activity, and DO NOT allow anyone to disclose this information.*

1. State that this was designed to help them think about what it might be like to discuss intimate information.

☑ *Ask if anyone felt uncomfortable about this possible disclosure? Listen and confirm responses. Next, ask what other questions or issues participants had as they thought about disclosure.*

2. If it is not named, it is important to ask if *issues about their colleague keeping information* ***confidential*** were a concern. Listen and confirm reactions.

3. State that, while all health facilities are committed to patient confidentiality, because of the significance of information shared in HIV testing, serious thought must be given to potential breaches of confidentiality and thought must be given about how to prevent problems.

☑ *Instruct participants to find a partner with whom they can work for 5 minutes. Once pairs have formed, ask partners to discuss the question: "Where are potential situations or places in the clinic when a patient's confidentiality may be compromised?"*

☑ *Advise them that they are not to mention any staff persons by name, but rather to focus on a generic discussion about potential pitfalls within the system where they work. Allow 5 minutes for discussion, and then invite the group to process the question.*

☑ *Ask the group to make a list of potential breeches of confidentiality that they discussed with their partners. Record these on flipcharts labeled "Possible Problems". Once all the potential problems have been identified, quickly divide group into smaller groups of 6-8 participants. Ask them to look at the list of potential problems and pick at least two that they can suggest possible safeguards for or strategies to implement to avoid problems.*

☑ *Allow 8-10 minutes for groups to discuss, and then invite discussion. As groups offer suggestions, write possible solutions beside the problem on the flipchart. Listen and confirm all responses. If any potential problems have not been addressed, facilitate a group discussion and create suggestions for these problems. Once this process is done, review any additional points as necessary. Possible solutions may include:*

- staff training about confidentiality,
- obtaining explicit permission to discuss patient's case with colleagues
- secure record-keeping and storage and other issues which are important to a confidential encounter.

4. Once you have finished, briefly refer back to the '3 C's' and point out that the second 'C' is informed consent. Explain that, in many health care settings, a patient's decision to come for care implies consent. Note that, because of the serious nature of HIV testing and counseling, it is ethically imperative that a provider determine that a patient consents to

testing and is rationally able to do so. To administer an HIV test without appropriate consent would be a grave error in judgment and a violation of a patient's rights.

5. Point out an important shift (which differs from voluntary HIV testing) in provider-initiated testing is a move to offering HIV testing with an 'opt-out' rather than an 'opt-in' philosophy. Note that this is a difference between the approach in voluntary counseling and testing (VCT).

☑ *Ask if anyone can distinguish between the two philosophical approaches. Listen and confirm responses, stressing that, in an 'opt-out' approach, the recommendation is made with the spirit of 'unless you object'.*

6. State that this shift may present a potential challenge for providers: the task of assuring voluntary consent while recommending HIV testing.

☑ *Ask participants to take a moment to think through possible ways to determine consent while recommending HIV testing. Listen and confirm responses. Add appropriate possible questions like:*

- *"Unless you object, I will get a sample for HIV testing. Routinely at the healthcare site, we offer everyone an annual HIV test. I think it will be important for you to know this information."*
- *I want to perform an HIV test today. If that isn't all right, you need to let me know."*
- *"I think this test will help me take care of your health/baby and, unless you object, I'm going to obtain a sample. Can you agree with me?"*

7. Point out that, in an 'opt-out' recommendation, advice is given and consent is obtained in a manner similar to ones discussed. Point out that an important additional suggestion may be to briefly include a provider's rationale or medical reason for suggesting HIV testing.

8. **Stress that, according to WHO/UNAIDS policy, the minimum amount of information required to provide informed consent includes:**

- **the clinical benefit and prevention benefits of testing**
- **the right to refuse**
- **the follow-up services that will be offered and**
- **the importance of persons who test positive informing past and future partners**

NOTE: You may wish to have "WHO/UNAIDS Policy on HIV Testing and Counselling" prepared as a supplemental handout.

9. Transition to the next section of the course, stating that this step--the provider recommending HIV testing--is one component of a four step model for provider-initiated HIV testing which is the foundation for the remainder of the course.

MODEL FOR PROVIDER-INITIATED HIV TEST INTERVENTION

Time: 15 minutes

Materials: Flipchart; Protocol For Provider-Initiated HIV Testing And Counseling (Slide 6)

Steps:

☑ *Reveal Slide 6.*

☑ *Quickly review the steps, using the following to guide the discussion.*

- **Step 1: Group Education session is given.** (Optional dependent on staff and time). A brief explanation about HIV transmission, HIV testing procedure, benefits of testing and prevention measures may be done. Note that this is a simpler and shorter intervention than normal individual pre-test counseling performed in VCT. Also, in some settings, this pre-test information may be given by another worker in the clinic in lieu of group education sessions.

- **Step 2: Provider engages patient individually and recommends HIV test, assures confidentiality, and obtains consent.**

  Remind participants about the 3 C's, stressing that in this step the provider focuses on voluntary consent and assuring confidentiality. Note that, where group education is not delivered, the educational components about HIV transmission and testing should be covered in this step.

- **Step 3: Specimen for HIV testing is obtained.**

  HIV testing will be performed utilizing the National HIV rapid testing algorithm. The specimens will be obtained through finger prick. Results will be provided to patients on the same day as testing. It is the responsibility of all participants in this training to be competent in the use of HIV Rapid Test Kits.

- **Step 4: HIV test results delivered to patient and referrals made**

  In step 4, the patient is given their HIV test results, those results are explained, and referrals for ongoing support or other needs are made. While the results may be given by any team member, preference will be placed on the more experienced provider, but not limited to them, when this is feasible. Note here that the test was recommended by a health care provider with a distinct medical rationale for it. It is logical that these results be interpreted--and follow up plan of care as indicated--by the health care provider who began the process.

☑ *Invite any final questions. State that the activities will shift to the next module, focusing on the skills to achieve step 2: brief pre-test information.*

END OF MODULE 2

MODULE 3: PATIENT EDUCATION STRATEGIES (MAY LIMIT TO TRAINING OF TRAINER SESSIONS)

Total Time: 40 minutes

15 minutes	Strategies for Effective HIV Education
10 minutes	Key Concepts: HIV Antibodies
15 minutes	Elements of Pre-Test Education

Module Objectives: Participants will be given basic information about good health education practices as they deliver HIV pre-test information. Focus will be on pertinent information related to pre-test information in provider-initiated testing intervention, including concepts of antibody development, and review of HIV testing criteria.

Preparation: Plan to use slide projector or flipcharts to augment lecture [Slide 7 and Slide 8].

STRATEGIES FOR EFFECTIVE HEALTH EDUCATION – FOCUS ON PATIENTS WHO OPT-OUT OF TESTING

Time: 15 minutes

Materials: Flipchart; Strategies for Effective Health Education (Slide 7)

Steps:

1. State that one important point in HIV testing will always be offering patients accurate information given by practitioners who are effective at patient education.

☑ *Reveal Slide 7. Use the following suggestions as your talking points.*

- **Address your patient's chief concern first.**
  Ask if anyone has ever been to the market when someone was trying to sell them something that they weren't interested in buying. Acknowledge responses and ask what they were thinking as this person continued to describe items they did not want to buy. Make the connection that when we don't offer education in a client-centered way, we run the risk that our patients feel the same way the participants felt in the market.

  Explain that, while we have information and expertise clients might need, our abilities to convey information are maximized if we begin by finding out our patient's agenda. State the simplest ways to learn patient's chief concern is to ask. Questions like: "What are you most concerned about?" or "What would you like to get out of today's visit?" are excellent examples.

- **Start education by asking what your patient already knows.**

One simple strategy for assessing a patient's information gaps as well as verbal abilities can be to ask an open question like "What do you know about the risks for HIV in this village/your community?" A patient's response gives you valuable information about their existing knowledge, and allows providers to work more efficiently by not teaching patients what they already know.

- **Use simple, non-technical language; use terms your patient uses.**
  Note that one ongoing challenge for health care workers is translating medical knowledge into concepts easily understood by patients. Stress that most providers do this all the time, but ask if anyone has ever realized that their patient hadn't understood what they said. Ask how they knew their patient had not comprehended. Listen and confirm responses.
  Ask what providers can do to increase the chances that the information we are trying to convey has been received. Listen and confirm strategies. Remind participants that one of the most certain ways to be sure you will be understood is to use language that is identical to the language patients use.

- **You can sometimes provide health education on sensitive topics by discussing them in third-person language.**
  One strategy with shy patients or uncomfortable topics may be to say something like, "many of our patients find it really hard to disclose their status to their husbands/partners." Ask participants what benefit might come from making statements in the third person terms. Listen and confirm responses like minimizing confrontation, decreasing anxiety, allowing patient to maintain confidentiality.

  State that the goal of health education is for patients to use information, and presenting information in a minimally threatening way is ideal. Note also that these kinds of interventions can also be important prompts for clients to discuss barriers to behavior change or feelings and issues which are getting in their way.

- **Patients can retain an average of only three take-home messages in any health education intervention.**
  Ask if anyone has ever needed to be taught a new skill by someone who was trying to offer them too much information and too many choices. Confirm responses. Explain that health information is as foreign to our patients as that information may have been for many of us.

  The skill for providers is to prioritize which health education issues are most important at this visit and to prioritize those while allowing the opportunity to return to other topics in subsequent visits.
  Share the observation that an important concept in behavior change is self-efficacy, a patient's ability to feel 'I can do that new behavior.' If patients are overwhelmed or given a message that there are many things they need to work on, the provider may actually decrease self-efficacy and increase feelings of pessimism about behavior change.

2. Transition to the next section by suggesting that the course now invites participants to discuss key educational concepts for providers performing HIV testing.

KEY CONCEPTS: HIV ANTIBODIES (MAY LIMIT TO TRAINING OF TRAINER SESSIONS)

Time: 15 minutes

Materials: Flipchart

Steps:

1. State that, in doing pre-test education in a testing context, certain information about HIV antibody development as well as prevention issues are critical.

2. Draw a horizontal line on a piece of newsprint, and make a mark near the left side of the line and label the mark 'HIV infection'. Suggest that, in order to be able to explain key concepts, you will be walking participants through a timeline of HIV infection with an emphasis on teaching points for clients taking HIV tests. Explain that this first mark signifies when someone is infected with HIV.

☑ *Ask what must have happened in advance of this event if we know that someone has been infected with HIV. Listen and confirm responses, focusing on two points: someone has been exposed to the infected fluids of another person and those fluids have had access to the other individual's blood system.*

☑ *Ask which fluids—outside of a medical setting—could have contributed to this individual becoming HIV infected. Listen and confirm the following fluids: blood, semen, vaginal secretions, and breast milk. Emphasize that, if someone was definitely infected with HIV, we would know they had to have been exposed to one of these fluids.*

3. Next, make a mark on the flipchart slightly to the right of the previous mark, and label this point 'Seroconversion'.

☑ *Ask participants what is meant by the term seroconversion. Listen and confirm responses, explaining that seroconversion refers to a period when enough antibodies have been produced that they can be detected by an antibody test. Make the point that an infected person can transmit the virus even before there are sufficient antibodies for a positive test.*

☑ *Ask participants how long it typically takes between HIV infection and development of a sufficient amount of antibodies for an HIV test to be reactive as a positive test. As participants respond, suggest that* ***most*** *individuals convert between 6 and 12 weeks after infection. State that almost everyone who has been exposed converts to antibody positive by six months.*

4. Draw a bracket between the existing points on the flipchart and label the space in between these points 'Window Period'.

☑ *Ask participants what is meant by this term. Confirm that the term window period is used to describe the time in between infection and seroconversion. Follow up by asking the implications of this on client education in a pre-test information session. Confirm response, highlighting the need to assess a patient's recent HIV risk behavior and instruct them on the need for re-testing of persons with possible recent exposure.*

☑ *Remind participants that virtually everyone infected with HIV will seroconvert within six months following exposure. Advise participants that, in doing pre-test information giving, emphasis should be placed on re-testing if possible exposures occurred less than six months before the current test.*

5. Invite any additional questions about the timeline and thank participants for their attention and involvement. Transition to the final activity of the module by stating that you will now cover the minimum expectations of pre-test information giving in provider-initiated testing.

PRE-TEST INFORMATION AND POST-TEST COUNSELING

Time: 15 minutes

Materials: Flipchart, Elements of Pre-Test Information (Slide 8)

Steps:

1. Begin this section by suggesting that there are several core components of HIV pre-test information giving which should be considered minimum standards in the testing intervention.

☑ *Reveal Slide 8, Elements of Pre-Test Education, and use the following points to guide the discussion.*

PRE-TEST

- **Mechanics and logistics of testing and receiving results-**
  Explain how the HIV screening and confirmatory rapid tests will be performed and the implications for waiting for results. If performing rapid tests, explain availability of results using national testing algorithm and policy to guide you.

  When clients will be waiting to receive their results, provide post-test card and information.You may wish to advise them of services available to clients of the healthcare setting during this time.

- **Facts about transmission and prevention (Provide in Group pre-test information dissemination or post-test during individualized sessions)-**
  State that providers performing HIV test must review ways HIV can be transmitted and strategies to avoid infection [or re-infection/co-infections for PLWHA]. Remind participants of the previous discussion of health education strategies and suggest the most effective way to begin this dialogue may be to ask clients what they have heard about HIV or about how people avoid contracting HIV.

- **Possible need for repeat testing-**

Remind participants of the previous section in this module with its emphasis on antibody development and the window period after infection. State that, at a minimum, providers should briefly explain that the accuracy of the current test will hinge on avoiding recent risk behavior.

POST-TEST

- **Information about availability of treatment and care services if positive-**
  Note that the rationale for scaling up HIV testing is intricately linked with the scale up of HIV treatment in the region. Advise participants that assuring clients of the availability of medical as well as social support services for persons living with HIV/AIDS must be done during the pre-test information giving. Stress that this implies providers will be proficient at knowing where resources are and committed to remaining knowledgeable as services are added and change in their communities.

- **Importance of disclosure –**
  Finally, it must be noted that it is important public health practice that the sexual and drug-sharing partners of persons testing HIV positive be advised of the possibility of their exposure to HIV, and the need for testing.

  Stress that the most helpful way to present this to a client is to emphasize that this service is always performed by professionals who assure their confidentiality. If clients are resistant, ask them to consider someone they love and the possibility their sexual partners had a communicable disease. Ask if they would want their loved one informed of the possible need to be tested.

  Lastly, stress that PLWHA should notify their health workers of their HIV status when they go for care or return to care. Emphasize that health care workers must maintain confidences, and that honestly informing health workers is primarily for the client's protection, not merely to protect the worker.

2. Invite any final suggestions about pre-test education components and thank participants for their interest. Transition to the next module by suggesting the course will now focus on a very challenging task in HIV testing: the giving of HIV test results.

END MODULE 3

MODULE 4: GIVING HIV TEST RESULTS AND EFFECTIVE REFERRALS (INCLUDE INFORMATION ON THE HIV HOTLINE)

Total Time: 55 minutes

15 minutes	Delivering HIV Test Results
20 minutes	Result Giving Practicum
20 minutes	Steps in Effective Referral

Module Objectives: Participants will be given a model for delivering HIV test results, and a chance to practice in simulation. Strategies for successfully referring patients into HIV testing and care as well as other services will be addressed.

Preparation: Flipchart; Delivering HIV Test Results (Slide 9) and Steps in Effective Referral (Slides 10-11).

DELIVERING TEST RESULTS

Time: 15 minutes

Materials: Flipchart, Slide 9

Steps:

1. The first task for health care workers offering HIV results is to think about the emotional responses patients may have to HIV test results.

☑ *Ask participants what emotions they have seen in patients **whose test result is negative.** Listen and confirm responses, adding any from your clinical practice. Expect answers like:*

- Relief (at not having HIV)
- Excitement (elation at good news)
- Confusion (may have perceived themselves as positive; may have positive current or former partners)
- Optimism (may feel like a new opportunity)

☑ *Next, invite the group to consider the emotional reactions for patients **whose test result is positive.** Again, confirm all responses, and record them on flipchart. Highlight these possible emotional responses to bad news:*

- Confusion
- Anger
- Denial
- Sadness

- Loss
- Bargaining
- Uncertainty
- Fear of death
- Shame/embarrassment
- Fear of rejection
- Disbelief

☑ *Ask participants to view the list of possible emotions and identify the ones they expect they would have if they tested HIV positive. Ask for volunteers to respond. If they need another prompt, ask "Which of these would be your reaction to hearing someone tell you that your test was HIV positive?" Invite as many participants to share their reactions as desire to do so. Thank each one without additional comment.*

☑ *Once this is finished, review newsprint from earlier in the day on which participants discussed the qualities they would hope for in a health care provider labeled "Helpful Traits". Ask participants which of these traits would be important to them as they consider their emotional reaction to HIV test results. Confirm all responses.*

2. Explain that sensitivity to patient emotions coupled, accurate medical information, and helpful referrals are the cornerstones of effective HIV result giving.

☑ *Reveal Slide 9, and discuss steps for delivering HIV test results:*

- **Verify patient identity:** Ensure that the person to whom you are giving the result is the same person who submitted for the test. Re check name, code, or identifying number or whichever was presented for testing.

- **Assess patient readiness to receive results:** Note that most patients are completely ready to hear the news of their results, and this should not drag out the waiting time. Rather, a short check in [i.e. 'Are you ready for your results?'] allows the patient to control this process and offers an opportunity for any additional questions or information.

- **Deliver and interpret HIV test result:** Promptly deliver the result, offering explanation of the test's meaning. One effective strategy can be, "The test result is _____ that suggests you DO/DO NOT have HIV in your blood system." This informs patient of the results, but also doesn't rely exclusively on use of terms reactive/non-reactive or even the terms positive and negative which may be confusing.

- **Allow for emotional reaction:** It can always be helpful to allow some time for silence after giving news, particularly a positive result. Offering an empathic comment [e.g. "This is really hard news to hear."] offers your patients a chance to talk about their emotions, perhaps to have them validated. If you have time and feel comfortable, use of open-ended

questions about their feelings is an excellent method for supporting patients.

- **Provide follow-up teaching/medical information as appropriate:** It may be important to remind patients about recent exposures and the need to be re-tested if they receive a negative result. Patients who test positive should be counseled about the need for medical follow up, availability of additional support services, and the need to notify sexual partners so they can also obtain testing.

  Note: Recent reports on patient with current STI and recent HIV exposure suggest that these patients may be especially infectious because of high levels of viremia following exposure. Particular emphasis on detection of acute HIV infection and education on need for re-testing is especially important in light of these findings.

- **Offer referrals and follow-up options:** Knowing about community/Hospital-based resources to address needs, especially of persons who tests positive, are critical for quality HIV testing centers. Reminding patients of additional services at your health center is important.

RESULTS GIVING PRACTICUM

Time: 30 minutes

Materials: N/A

Steps:

1. State that the course will now offer a chance for participants to practice delivering an HIV test result.

☑ *Ask participants to partner with one person in the group with whom they have not worked during the training. Once groups have formed, tell the person whose birthday is closest to January 1 that they will play patient in this simulation. Ask them if they are willing to be patients who have been tested in the health center. If so, ask them to step out of the room while you instruct the health care workers.*

☑ *Participants left will be health care providers. Advise them that they get to choose which results to practice. State the suggestions of the previous section should guide this intervention. They should imagine the patient had left briefly after the test is conducted. The patient will be led back in just a moment to hear test results. Again, remind provider they should determine if they would like to give a positive or negative result in this practice session.*

☑ *Bring patients back into the training room, and advise them that their test results have been read and their provider is prepared to discuss them. Instruct them that some sessions may take longer, so if they finish before 10 minutes, they should quietly discuss the session with their provider. Ask providers to begin result-giving simulation. Monitor participant's work, and after 8-10 minutes end the simulation.*

☑ *Ask patients to debrief with their provider by focusing on all the 'successes' they can find. Instruct them to focus on the specific details of what providers did well. Invite participants to discuss among themselves what went well initially, and advise them the group will have a discussion after they have concluded.*

☑ *Allow five minutes for partners to debrief, and then ask the group what went well. Ask particular participant what felt good to them when they were given results. Invite counselors to discuss their reactions to this process, and what things they felt worked effectively.*

☑ *Once discussion has ended transition to the next activity by asking providers if they felt the patient they were seeing could benefit from additional community services or medical care. Next, ask how often real patients will need support or medical services from the HIV testing center. Acknowledge this common occurrence. State that the task is so important, we take some time to focus on steps for making effective referrals.*

STEPS IN EFFECTIVE REFERRAL

Time: 20 minutes

Materials: Steps in Effective Referral (Slides 10-11)

Steps:

1. Begin the module by emphasizing that one of the most important components of post-test support in the provider-initiated testing session must be the process of making referrals. Emphasize this is especially important for persons who test positive.

☑ *Reveal Slide 10 (continue with Slide 11) and offer some suggestions for effective referral, emphasizing that the goal is to maximize that a patient follows through with our referral.*

- **Referral letter-** A referral letter establishes authority and assists the client to act on their own behalf. It should be noted that a referral is not a way to pass a problem along, but has a meaningful purpose for promoting the clients well being.

- **Observe Confidentiality-** In the course of making referrals, confidentiality must be observed. Patients should know the information about themselves that is included in making the referral. The referral letter which you are giving to the patient should not contain any information that you would not want them to know or can prove harmful to them if it is lost or misplaced. Agency guidelines should direct the nature and content of information to be included on referral forms.

- **Maintain and Update Directory –** Formalizing the process of development of a directory and charging one individual with responsibility to maintain this resource facilitates easy access to a listing of multiple services and programmes for persons who need various forms of support and assistance. Know the resources that are available in your environment and use them.

- **Refer to known and trusted resources-** The most effective referrals are to specific providers with whom you have had experience and who can deliver.

- **Offer referral as one option-** In a client-centered approach, the client is offered the referral as a possible resource for them to take advantage of. It is contradictory in a humanistic approach to attempt to mandate clients receive any service: this is both disempowering and holds the possibility of clients reacting to our directives by rejecting them.

- **Assess client's reaction to referral-** Note that clients may have a history with agencies and providers, and some referrals may have negative stigma in client's minds. It is the client's right to refuse a referral, it is your responsibility to offer.

- **Instruct clients and prepare clients for agency services you are referring to-** Suggest that specific teaching about what to bring, how to get to agencies, and what to expect at the organization will maximize the possibility that clients don't 'fall through the cracks' of the service plan.

- **Assess level of support for active referral-** At times, client empowerment means giving client little direction and encouraging them to follow through, while other clients need to have appointments made or a plan of assistance developed. Managing the balance of providing too much support or not enough support is a critical clinical choice.

- **Follow-up with client and referral source as appropriate:** determine mechanism to find out from client and/or provider if client did follow through and how successful the interaction was.

- **Receiving referrals**- This is an equally important part of the referral process. How you receive and treat with a referred client will determine the success or failure of the intervention. It is important to acknowledge receipt, preferably through a written response. Where possible you may also wish to make a quick phone call to the referring agency just to inform that the referral has been used. All agencies should establish follow up and evaluation procedures.

- **Disclosure and partner notification:** Your role is to assist clients who are HIV positive to understand the importance of partner notification and the importance of their present and past sex partners being tested if this has not already been done. It is imperative that providers assist clients to develop a plan for disclosure. In addition, interaction with the Public Health System in Jamaica, utilizing the Class1/HIV confidential reporting forms, will activate the Contact Investigator system and allow for anonymous tracing.

It is suggested providers allow clients to discuss their fears and ask questions about any critical challenges that they may face, such as violence; this can direct where and to whom you refer.

Explore possible immediate personal sources of support such as friends or family. Referral to an identified professional, support group, or agency, can be useful for providing support and coaching in disclosure and partner notification.

2. Invite any final recommendations and suggestions for making referrals. Listen and confirm any responses.

3. Finally, acknowledge that the primary care provider may not have time to perform all the necessary functions associated with making referral. Ideally, the primary care provider would be the person who explains and emphasizes the need for and availability of treatment and care. In any event, the chief concern for the interdisciplinary team must be an assessment of appropriate referrals for clients receiving HIV test results and employing clinical skills at encouraging client follow-through on referrals made.

4. Transition to the workshop closure by explaining that the content for the workshop has been completed, and the final steps in the workshop will begin.

END OF MODULE 4

WORKSHOP CLOSURE

Time: 10 minutes

Materials: Evaluations

Steps:

- ☑ *Announce to participants that the training is concluding. Thank them for their hard work and willingness to participate.*

- ☑ *Reveal newsprint labeled:*
  "I learned
  or
  One thing I'll do differently".

- ☑ *Ask if anyone wants to conclude the experience by sharing something they learned that was meaningful or something they will do differently. Invite participants who are willing to mention one thing they found valuable if they choose.*

1. When participants have finished, instruct them to complete evaluation, discuss any follow-up training opportunities, and adjourn the workshop

END OF WORKSHOP